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Child Health Division · Ministry of Health and Family Welfare

HOME BASED NEWBORN CARE (HBNC) OPERATIONAL GUIDELINES

Operational Field Protocol for Reducing Neonatal Mortality

DOCUMENT ID: R6-NHM-2026 **EDITION:** Official Field Standard 4.2 **TARGET:** Frontline ASHA Workers

1. Core HBNC Objectives for Frontline ASHA Workers

• Early identification of illnesses in the newborn and provision of timely care/referral.
• Promotion of exclusive breastfeeding within the first hour of birth.
• Prevention of hypothermia through Kangaroo Mother Care (KMC) and appropriate wrapping.
• Supporting mothers in infection prevention through clean umbilical cord and skin care.

2. Clinical Danger Signs in the Newborn

• Inability to feed / poor suckling.
• Hypothermia (body temp < 35.5°C / feet cold to touch) or Fever (temp > 37.5°C).
• Chest indrawing / fast breathing (> 60 breaths per minute).
• Lethargy, unconsciousness, or abnormal convulsions.
• Jaundice appearing within first 24 hours of birth or extending to palms and soles.

3. Low Birth Weight (LBW) & Kangaroo Mother Care Protocol

• Any infant weighing < 2500 grams must be screened for specialized domiciliary support.
• Promote Kangaroo Mother Care (continuous skin-to-skin contact with mother's chest) for at least 8-12 hours daily.
• Ensure frequent, on-demand feeding every 2 hours.

Issuing Authority:
National Health Mission Division
Nirman Bhawan, New Delhi

Field Protocol Verification:
Primary Health Centre / Block MO
Mandatory compliance across sub-centres

ASHA Frontline Desk:
Document all actions in MCP Card
Follow 108 Emergency Protocol